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Time taken	15 secs
Marks	0/73
Grade	0 out of 100



Question 1

Not answered

Marked out of 1

The term that refers to the neglect of all alternate forms of pleasure apart from alcohol is

Select one:

- ☐ Narrowing of repertoire
- ☐ Salience
- ☐ Dependence
- ☐ Reinstatement
- ☐ Tolerance

Your answer is incorrect.

Explanation:

Salience (or primacy) of alcohol-seeking behaviour refers to the neglect of all leisure and alternate forms of pleasure apart from alcohol. Alcohol becomes more important than retaining a job or relationships or remaining financially solvent and in good physical health. It may also diminish the moral sense, leading to criminal activity and fraud.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 574.

Edwards G, Gross MM. Alcohol dependence: provisional description of a clinical syndrome. British medical journal. 1976 May 1;1(6017):1058.

The correct answer is: Salience



Question 2

Not answered

Marked out of 1

Which of the following is a feature of type 1 alcoholism, according to Cloninger?

Select one:

- ☐ High antisocial traits
- ☐ Strong heritability
- ☐ Male predominated
- ☐ Inability to abstain
- ☐ Starts after 25 years

Your answer is incorrect.

Explanation:

Cloninger's classification:

- **Type 1 ('late onset alcoholism')**: individuals who become dependent after the age of 25, have loss of control, not much criminality, and no strong family history; both males and females
- **Type 2 ('early onset alcoholism')**: individuals who become dependent before the age of 25, have inability to abstain, antisocial personality traits, and a family history of alcoholism; often male

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on diagnosis, assessment and management of harmful drinking and alcohol dependence. Updated edition 2020.

The correct answer is: Starts after 25 years

Question 3

Not answered

Marked out of 1

Which of the following statements about alcoholic hallucinosis is incorrect?

Select one:

- ☐ The hallucinations usually last less than a week
- ☐ Patients have a clear sensorium
- ☐ The most common hallucinations are unstructured sounds or derogatory voices
- ☐ The hallucinations persist and respond poorly to antipsychotics
- ☐ The hallucinations typically occur in persons abusing alcohol for a long time

Your answer is incorrect.

Explanation:

Alcoholic hallucinosis is a substance-induced psychotic illness that is a rare complication of prolonged heavy alcohol use. In ICD-11, it is designated as 'alcohol-induced psychotic disorder with hallucinations'. It is characterised by experiencing hallucinations (usually auditory) in clear consciousness and while sober. It usually starts with elemental hallucinations (e.g. bangs or murmurings). With continued alcohol use they become formed voices, usually derogatory. In 95% of patients, symptoms rapidly resolve on ceasing alcohol consumption, but rapidly recur on restarting drinking. In approximately 5%, there are prolonged symptoms. Persisting symptoms can be treated with antipsychotics.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 602.

International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

The correct answer is: The hallucinations persist and respond poorly to antipsychotics

Question 4

Not answered

Marked out of 1

Which drug should be given as prophylaxis against irreversible neurological damage in all patients undergoing inpatient alcohol detoxification?

Select one:

- ☐ Diazepam
- ☐ Diphenoxylate
- ☐ Naltrexone
- ☐ Thiamine
- ☐ Acamprosate

Your answer is incorrect.

Explanation:

It is generally advised that patients undergoing inpatient alcohol detoxification should be given parenteral thiamine (vitamin B complex; in the UK – Pabrinex) as prophylaxis. The standard advice is one pair of Pabrinex IM high potency daily (containing thiamine 250mg/dose) for 5 days, followed by oral thiamine and/or vitamin B compound for as long as needed (where diet is inadequate, or alcohol consumption is resumed). All inpatients should receive this regime as an absolute minimum.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 461-462.

The correct answer is: Thiamine

Question 5

Not answered

Marked out of 1

In the UK, what is considered as a safe limit of weekly alcohol consumption in women?

Select one:

- ☐ 5 units
- ☐ 14 units
- ☐ 21 units
- ☐ 30 units
- ☐ 10 units

Your answer is incorrect.

Explanation:

New revised low-risk drinking guidelines were published UK-wide in 2016. They proposed that to minimise health risks from alcohol, a limit of 14 units per week for both men and women who drink regularly is recommended. This constituted a reduction for men, the previous limit being 21 units per week. For those drinking up to 14 units, spreading drinking over 3 or more days is advised. Drink-free days per week are also recommended. No alcohol during pregnancy is advised as the safest approach.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 586.

The correct answer is: 14 units

Question 6

Not answered

Marked out of 1

The amount of alcohol units in two pints (2 X 568ml) of beer that has a strength of 4% alcohol by volume (ABV) is

Select one:

- ☐ 10U
- ☐ 2 U
- ☐ 4.5U
- ☐ 1 U
- ☐ 7 U

Your answer is incorrect.

Explanation:

The amount of alcohol in drinks is measured in **units**. One unit contains approximately **8g of alcohol**. In alcoholic drinks where the percentage of alcohol by volume is given:

Number of units = volume in litres x % of alcohol

Thus two pints (2 X 568ml) of beer at 4% ABV contains $1.136 \times 4 = 4.5\text{U}$

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 584.

The correct answer is: 4.5U



Question 7

Not answered

Marked out of 1

The symptoms of alcohol withdrawal usually subside within how many days after cessation of alcohol use?

Select one:

- ☐ 1-2 days
- ☐ 10-12 days
- ☐ 2-3 days
- ☐ 14 days
- ☐ 3-7 days

Your answer is incorrect.

Explanation:

Features of mild or moderate alcohol withdrawal typically last for 3-7 days after cessation of alcohol use and include autonomic hyperactivity, increased hand tremor, anxiety, insomnia, nausea, vomiting, and headache.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

The correct answer is: 3-7 days



Question 8

Not answered

Marked out of 1

According to NICE guidelines, opioid detoxification with buprenorphine in an inpatient setting may last for

Select one:

- ☐ 3-5 days
- ☐ 5-7 days
- ☐ 7-14 days
- ☐ 14-21 days
- ☐ 21-30 days

Your answer is incorrect.

Explanation:

Detoxification in an inpatient setting, the NICE guidelines indicate, may take place over a shorter time than in the community (suggesting 14-21 days for methadone and 7-14 days for buprenorphine) 'as the supportive environment helps a service user to tolerate emerging withdrawal symptoms'. As in the community, a stabilisation on the dose of a substitute opioid is first achieved, followed by gradual dose reduction, with additive medications judiciously prescribed for withdrawal symptoms if and as needed.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 494.

The correct answer is: 7-14 days

Question 9

Not answered

Marked out of 1

Which of the following is regarded as the most suitable screening tool in primary care settings for detecting hazardous and dependent drinking?

Select one:

- ☐ Michigan Alcohol screening test
- ☐ Fast alcohol screening test
- ☐ Alcohol use disorders identification test
- ☐ Gamma glutamyl transferase levels
- ☐ CAGE questionnaire

Your answer is incorrect.

Explanation:

NICE recommends the Alcohol Use Disorders Identification Test (AUDIT), particularly for use by **staff who are not working in specialist alcohol treatment services** (e.g. GPs and acute hospital and mental healthcare staff).

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on diagnosis, assessment and management of harmful drinking and alcohol dependence. Updated edition 2020.

The correct answer is: Alcohol use disorders identification test



Question 10

Not answered

Marked out of 1

Which of the following is a 10-item questionnaire that is useful as a screening tool for hazardous or harmful alcohol use?

Select one:

- ☐ Motivational interview
- ☐ CAGE
- ☐ Clinical global impression scale
- ☐ Michigan alcohol screening test
- ☐ Alcohol use disorders identification test

Your answer is incorrect.

Explanation:

The AUDIT is a 10-item questionnaire which is useful as a screening tool in those identified as being at increased risk of alcohol-related problems. Questions 1-3 address the quantity of alcohol consumed, 4-6 the signs and symptoms of dependence and 7-10 the behaviours and symptoms associated with harmful alcohol use. Each question is scored 0-4, giving a maximum total score of 40. A score of 8 or more is suggestive of hazardous or harmful alcohol use.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 454.

The correct answer is: Alcohol use disorders identification test



Question 11

Not answered

Marked out of 1

A 34-year-old patient who has been dependent on alcohol is involved in a road traffic accident and admitted to an orthopaedic ward for multiple fractures. Which of the following is correct with respect to his clinical state?

Select one:

- ☐ Alcohol withdrawal symptoms usually start within 3-12 hours
- ☐ The chances of experiencing delirium is highest after 5 days
- ☐ He will not have a seizure unless he has a head injury
- ☐ Risk of seizures is maximal from day 3 to day 5
- ☐ Having withdrawal symptoms 5 days after abstinence is unusual

Your answer is incorrect.

Explanation:

Mild alcohol withdrawal symptoms usually start within 3-12 hours of the last drink and peak at 24-48 hours. They may last up to 14 days. Generalised seizures usually start within 12-18 hours of the last drink. They may commence before blood alcohol levels reach zero. Delirium tremens usually start 3-4 days after the last drink.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 454-455.

The correct answer is: Alcohol withdrawal symptoms usually start within 3-12 hours

Question 12

Not answered

Marked out of 1

Disulfiram inhibits which of the following enzymes?

Select one:

- ☐ Formaldehyde dehydrogenase
- ☐ Butyraldehyde dehydrogenase
- ☐ Acetaldehyde dehydrogenase
- ☐ Aldehyde dehydrogenase
- ☐ Alcohol dehydrogenase

Your answer is incorrect.

Explanation:

Disulfiram acts via the irreversible inhibition of aldehyde dehydrogenase, resulting in the accumulation of acetaldehyde. Signs and symptoms such as flushing, nausea, vomiting, headache, tachycardia and palpitations arise if alcohol is consumed. The reaction is dose-dependent; if a large amount of alcohol is consumed, a more severe reaction may lead to hypertension, collapse, and even death.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 598.

The correct answer is: Aldehyde dehydrogenase



Question 13

Not answered

Marked out of 1

Which class of drugs is recognised as the treatment of choice for alcohol withdrawal?

Select one:

- ☐ Stimulants
- ☐ Antipsychotics
- ☐ Antidepressants
- ☐ Mood stabilisers
- ☐ Benzodiazepines

Your answer is incorrect.

Explanation:

Benzodiazepines are first-line treatment for managing alcohol withdrawal. They reduce key symptoms, withdrawal severity, and the incidence of delirium and seizures. The most commonly used benzodiazepines are diazepam, chlordiazepoxide, oxazepam, and lorazepam.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 592-593.

Sinclair JM, O'Neill A. Update on the management of alcohol use disorders. BJPsych Advances. 2020 Mar;26(2):82-91.

The correct answer is: Benzodiazepines

Question 14

Not answered

Marked out of 1

Which one among the following is not associated with alcoholism?

Select one:

- ☐ Binswanger disease
- ☐ Primary alcoholic dementia
- ☐ Korsakoff syndrome
- ☐ Marchiafava-Bignami disease
- ☐ Hepatocerebral degeneration

Your answer is incorrect.

Explanation:

In Binswanger disease (Binswanger arteriopathic subcortical encephalopathy), numerous lacunar infarcts are present in the white matter and basal ganglia. This is typically seen in patients with long-standing hypertension. It is not linked to alcohol use.

Ref: Gray F, Duyckaerts C, de Girolami U. Escourolle and Poirier's Manual of Basic Neuropathology, 6th Edition. 2019. p. 119.

The correct answer is: Binswanger disease



Question 15

Not answered

Marked out of 1

In the UK, which one of the following is the drug of choice in patients with an uncomplicated alcohol withdrawal syndrome?

Select one:

- ☐ Temazepam
- ☐ Chlordiazepoxide
- ☐ Diazepam
- ☐ Clonazepam
- ☐ Oxazepam

Your answer is incorrect.

Explanation:

Benzodiazepines are the treatment of choice for alcohol withdrawal. In the UK, chlordiazepoxide is the benzodiazepine used for most patients in most centres as it is considered to have a relatively low dependence-forming potential. Some centres use diazepam. A short-acting benzodiazepine such as oxazepam or lorazepam may be used in individuals with impaired liver function.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 458-459.

The correct answer is: Chlordiazepoxide



Question **16**

Not answered

Marked out of 1

You are called to A&E to see a man withdrawing from amphetamines. He is least likely to have

Select one:

- ☐ Insomnia
- ☐ Hypersomnia
- ☐ Agitation
- ☐ Decreased appetite
- ☐ Psychomotor retardation

Your answer is incorrect.

Explanation:

Features of stimulant withdrawal (including amphetamines, methamphetamine, and methcathinone) may include dysphoric mood ('crash') sometimes with suicidal ideation, irritability, fatigue, vivid & unpleasant dreams, insomnia or (more commonly) hypersomnia, increased/insatiable appetite, psychomotor agitation or retardation, and craving for amphetamine or related stimulants.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

The correct answer is: Decreased appetite

Question 17

Not answered

Marked out of 1

Which of the following drugs should be supervised by a family member or a carer?

Select one:

- ☐ Chlordiazepoxide
- ☐ Oxazepam
- ☐ Acamprosate
- ☐ Diazepam
- ☐ Disulfiram

Your answer is incorrect.

Explanation:

The therapeutic effect of disulfiram is mediated by its incompatibility with alcohol, resulting in alcohol aversion. Supervised medication optimises compliance and contributes to effectiveness. According to NICE, disulfiram should be considered in combination with a psychological intervention for patients who wish to achieve abstinence, but for whom acamprosate or naltrexone is not suitable. Treatment should be started at least 24 hours after the last drink and should be overseen by a family member.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 465-466.

The correct answer is: Disulfiram

Question 18

Not answered

Marked out of 1

Which of the following is not a characteristic feature seen in hepatocerebral degeneration due to alcoholism?

Select one:

- ☐ Cognitive impairment
- ☐ Tremor
- ☐ Gait ataxia
- ☐ Asterixis
- ☐ Choreoathetosis

Your answer is incorrect.

Explanation:

Hepato-cerebral degeneration is a neurological syndrome consisting of various movement disorders and cognitive impairment in advanced liver cirrhosis. It is characterised by tremor, choreoathetosis, dysarthria, gait ataxia, and cognitive impairment. Orolingual dyskinesia, dystonia, and pyramidal tract signs may also be seen. Acute hepatic encephalopathy (characterised by asterixis, hyperventilation, EEG abnormalities, and alterations in consciousness) can occur before and after the onset of hepato-cerebral degeneration.

Ref: Shin HW, Park HK. Recent updates on acquired hepatocerebral degeneration. Tremor and Other Hyperkinetic Movements. 2017;7:1-12.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 261.

The correct answer is: Asterixis

Question 19

Not answered

Marked out of 1

Alcohol blocks the consolidation of new memories into old memories through its actions at which of the following brain regions?

Select one:

- ☐ Amygdala
- ☐ Hippocampus
- ☐ Pineal gland
- ☐ Mamillary bodies
- ☐ Periventricular aqueduct

Your answer is incorrect.

Explanation:

In alcohol induced amnesia (also called 'blackouts' or 'palimpsest'), there is a breakdown in transfer of information from short-term to long-term storage that occurs while immediate (very brief short-term) and remote (long-term; formed prior to intoxication) memory abilities remain intact. Alcohol facilitates faster forgetting. Rapid forgetting is a hallmark of hippocampal dysfunction.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 600.

Hermens DF, Lagopoulos J. Binge drinking and the young brain: A mini review of the neurobiological underpinnings of alcohol-induced blackout. Frontiers in psychology. 2018 Jan 19;9:12.

The correct answer is: Hippocampus

Question **20**

Not answered

Marked out of 1

Which one of the following is not a marker for excess alcohol consumption?

Select one:

- ☐ Gamma glutamyl transferase
- ☐ Carbohydrate deficient transferase
- ☐ Lactate dehydrogenase
- ☐ Mean corpuscular volume
- ☐ All the listed options are markers for excess consumption

Your answer is incorrect.

Explanation:

Elevated red cell MCV, GGT, and carbohydrate-deficient transferrin (CDT) are markers for excess alcohol consumption.



Markers of Alcohol Use / Disorders

Markers	Time to normalize on abstinence	Utility
Breathalyzer or Blood Alcohol Levels	6 hrs in blood; 12-24 hrs in breath	Highly specific; useful in assessing recent drinking (e.g. in supervised detox)
γ-glutamyl transferase (GGT)	4 weeks	Moderate specificity; low sensitivity; best used in follow-up
Mean corpuscular volume of red blood cells (MCV)	3-6 months	Moderate specificity; low sensitivity; best used in follow-up
Aspartate aminotransferase (or AST/ALT ratio)	4 weeks	Low specificity; low sensitivity
Carbohydrate-deficient transferrin (CDT)	4 weeks	High specificity; moderate sensitivity; can be used to monitor relapse; low availability
Urinary ethyl glucuronide (an alcohol metabolite)	Several days	Sensitive to ingestion of one or two drinks

Ref: Oxford Handbook of Psychiatry 2019. pp. 582-583.

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Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 582-583.

The correct answer is: Lactate dehydrogenase



Question **21**

Not answered

Marked out of 1

Which of the following types of alcohol-related neurological damage has an association with red wine?

Select one:

- ☐ Alcoholic dementia
- ☐ Hepatocerebral degeneration
- ☐ Alcoholic hepatic encephalopathy
- ☐ Cerebellar ataxia
- ☐ Marchiafava-Bignami syndrome

Your answer is incorrect.

Explanation:

CNS myelin is vulnerable to numerous toxins. For example, a contaminant of certain homemade Italian red wines probably causes degeneration of the heavily myelinated corpus callosum, causing the Marchiafava-Bignami syndrome. Theoretically, at least, this syndrome can damage the corpus callosum enough to produce a 'split brain syndrome'. Patients show disconnection signs and extensive cerebral dysfunction.

Ref: Kaufman DM, Geyer HL, Milstein MJ, Rosengard JL. Kaufman's Clinical Neurology for Psychiatrists, 9th Edition. 2023. pp. 129, 172, 356.

The correct answer is: Marchiafava-Bignami syndrome

Question 22

Not answered

Marked out of 1

A pregnant woman who has not been prescribed any medications presents with opioid dependence. Which of the following treatments should be initiated?

Select one:

- ☐ Naltrexone
- ☐ Methadone
- ☐ Acamprosate
- ☐ Buprenorphine
- ☐ Acute detoxification

Your answer is incorrect.

Explanation:

Women can present with opioid dependency at any stage in pregnancy, and stabilisation on substitute methadone is the treatment of choice. Both methadone and buprenorphine maintenance improve maternal and foetal outcomes, and substitution treatment should be offered to pregnant opioid-dependent women. Buprenorphine is associated with less severe neonatal withdrawal symptoms. However, buprenorphine should not be initiated in pregnancy because of the risk of inducing withdrawal in the foetus. There is little evidence for acute detoxification in pregnancy. Pregnant opioid-dependent women should be encouraged to access opioid maintenance treatment rather than attempt detoxification on the basis that relapse rates are high and maternal and foetal risks are greater from failed detoxification and relapse than from maintenance treatment.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 498.

McAllister-Williams RH et al. British Association for Psychopharmacology consensus guidance on the use of psychotropic medication preconception, in pregnancy and postpartum 2017. Journal of Psychopharmacology. 2017 May;31(5):519-52.

The correct answer is: Methadone

Question **23**

Not answered

Marked out of 1

Mr. X is a 45-year-old man admitted to hospital with features of severe alcohol withdrawal. Laboratory investigations are arranged. Which one of the following findings is least likely to be found in his blood test results?

Select one:

- ☐ Microcytosis
- ☐ Neutropenia
- ☐ Low haemoglobin
- ☐ Thrombocytopenia
- ☐ Raised liver function tests

Your answer is incorrect.

Explanation:

Commonly seen blood-based markers of excess alcohol use include anaemia, low platelets (thrombocytopenia), neutropenia, raised MCV (macrocytosis), and raised liver function tests.

Ref: Taylor DM, Gaughran F, Pillinger T. The Maudsley Practice Guidelines for Physical Health Conditions in Psychiatry. 2021. p. 219.

The correct answer is: Microcytosis



Question **24**

Not answered

Marked out of 1

Mr. W is a 46-year-old man being treated for opioid dependence who took an overdose of methadone. Which drug should be administered in the event of a methadone overdose?

Select one:

- ☐ Buprenorphine
- ☐ Naloxone
- ☐ Naltrexone
- ☐ Acamprosate
- ☐ Pabrinex

Your answer is incorrect.

Explanation:

Opioid overdose is a preventable cause of death. This includes overdose on illicit opioids such as heroin and more recently fentanyl and oxycodone, and overdose on prescribed opioids such as methadone or buprenorphine. Opioid overdose is characterised by the triad of unconsciousness, low respiratory rate, and pin-point pupils. Cyanosis and cold, clammy skin may also be seen. Naloxone is an opioid receptor antagonist that can reverse opioid overdose. It is available in pre-loaded syringes that should be administered intramuscularly. Higher doses of naloxone may be needed to displace opioids of high affinity such as buprenorphine or fentanyl. Recently, concentrated intranasal naloxone has been developed as an alternative to IM naloxone.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 474-475.

The correct answer is: Naloxone

Question **25**

Not answered

Marked out of 1

Mr. Smith used to drink at several places and consume various different drinks of his choice until five years ago. Now, he only drinks at home, sticking to vodka. He is exhibiting

Select one:

- ☐ Loss of control
- ☐ Relief drinking
- ☐ Salience
- ☐ Narrowing of the repertoire
- ☐ Neuroadaptation

Your answer is incorrect.

Explanation:

Narrowing of the drinking repertoire occurs when the user moves from consuming a range of beverages to drinking a single type of beverage in preference to all others. Dependent drinkers will also settle into a fixed setting. Narrowing of the repertoire may help them maintain a steady level of alcohol in the blood without much fluctuation.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 574.

Edwards G, Gross MM. Alcohol dependence: provisional description of a clinical syndrome. British medical journal. 1976 May 1;1(6017):1058.

The correct answer is: Narrowing of the repertoire

Question 26

Not answered

Marked out of 1

A 40-year-old woman with significant alcohol-related problems is not considering changing her behaviour or lifestyle. She denies that any of her problems are alcohol-related. She is in which of the following stages of change?

Select one:

- ☐ Contemplation
- ☐ Action
- ☐ Relapse
- ☐ Precontemplation
- ☐ Maintenance

Your answer is incorrect.

Explanation:

Six stages of change:

- (1) **Precontemplation:** not even considering changing behaviour; does not see the behaviour as a problem, minimizes and denies associated risks, and avoids information to the contrary
- (2) **Contemplation:** has become aware that the behaviour is a problem but is ambivalent about changing, and likely sees equal or more benefits than costs from the behaviour
- (3) **Preparation:** has made a decision to attempt change
- (4) **Action:** has implemented a plan and is changing the behaviour
- (5) **Maintenance:** maintaining gains made and attempting to improve areas of life that have been harmed
- (6) **Relapse:** return to previous behaviour, but with the possibility of gaining useful strategies to extend the maintenance period on the next attempt

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 575.

The correct answer is: Precontemplation



Question 27

Not answered

Marked out of 1

Which of the following features is suggestive of schizophrenia instead of alcoholic hallucinosis in patients who have psychotic symptoms on a background of alcoholism?

Select one:

- ☐ Onset of alcohol drinking clearly preceding the onset of psychosis
- ☐ Remission of psychosis during abstinence
- ☐ Atypical or late age of onset of psychotic symptoms
- ☐ Presence of thought disorder and affect incongruence
- ☐ Recurrence of hallucinations when drinking pattern is reinstated

Your answer is incorrect.

Explanation:

Alcoholic hallucinosis is a substance-induced psychotic illness that is a rare complication of prolonged heavy alcohol use. In ICD-11, it is designated as 'alcohol-induced psychotic disorder with hallucinations'. It is characterised by experiencing hallucinations (usually auditory) while sober. It usually starts with elemental hallucinations (e.g. bangs or murmurings). With continued alcohol use they become formed voices, usually derogatory. In 95% of patients, symptoms rapidly resolve on ceasing alcohol consumption, but rapidly recur on restarting drinking.

Evidence supporting a diagnosis of schizophrenia (or a mood disorder with psychotic symptoms) rather than alcoholic hallucinosis include psychotic symptoms preceding the onset of alcohol use, the symptoms persisting for a substantial period of time after cessation of alcohol use or withdrawal (e.g., 1 month or more), or other evidence of a pre-existing mental disorder with psychotic symptoms. Of note, some people use substances to suppress symptoms of mental disorders like schizophrenia and full symptomatic presentations only emerge upon cessation or reduction in substance use.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 602.

International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

The correct answer is: Presence of thought disorder and affect incongruence



Question 28

Not answered

Marked out of 1

Mr. X is a patient who had been alcohol abstinent for a long time but went back to his original pattern of heavy drinking in a surprisingly rapid manner after having a priming dose of alcohol. This is referred to as

Select one:

- ☐ Abuse
- ☐ Compulsion
- ☐ Reinstatement
- ☐ Harmful use
- ☐ Demotivation

Your answer is incorrect.

Explanation:

'Reinstatement after abstinence' is a criterion stressed by Edwards & Gross in their early description of alcohol dependence. When a user relapses to alcohol use after a period of abstinence, they are at risk of a return to the dependent pattern in a much shorter period than the time initially taken to reach dependent use. Explanations for this include:

1. **Abstinence violation effect** – the cognition that 'I had a drink, so I am a drinker again' may cause patients to reinstate a full pattern of drinking
2. **P propensity to experience withdrawal symptoms** may be carried over even through the period of abstinence in some individuals
3. **Cues may trigger memories after a priming dose**, precipitating return of compulsive drinking

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 574.

Edwards G, Gross MM. Alcohol dependence: provisional description of a clinical syndrome. British medical journal. 1976 May 1;1(6017):1058.

The correct answer is: Reinstatement

Question 29

Not answered

Marked out of 1

Which of the following is a feature of type 2 alcoholism, according to Cloninger?

Select one:

- ☐ Milieu limited
- ☐ Lack of strong family history
- ☐ Lack of antisocial traits
- ☐ Seen usually in males
- ☐ Later age of onset

Your answer is incorrect.

Explanation:

Cloninger's classification:

- **Type 1 ('late onset alcoholism')**: individuals who become dependent after the age of 25, have loss of control, not much criminality, and no strong family history; both males and females
- **Type 2 ('early onset alcoholism')**: individuals who become dependent before the age of 25, have inability to abstain, antisocial personality traits, and a family history of alcoholism; often male

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on diagnosis, assessment and management of harmful drinking and alcohol dependence. Updated edition 2020.

The correct answer is: Seen usually in males

Question 30

Not answered

Marked out of 1

Diminished or reverse tolerance is associated with all of the following except

Select one:

- ☐ Idiosyncratic in certain individuals
- ☐ Young female drinkers
- ☐ Metabolic problems such as liver disease
- ☐ When drinking alcohol after a period of abstinence
- ☐ Older patients with brain damage

Your answer is incorrect.

Diminished tolerance is a notable phenomenon in drinkers. When drinking alcohol after a period of abstinence, the tolerance may revert to normal and might lead to quick intoxication. Older patients with brain damage may have curtailed neuroadaptation leading to reversed tolerance. Metabolic problems such as liver disease may apparently look like diminished tolerance. It may also be idiosyncratic in certain individuals.

The correct answer is: Young female drinkers

Question **31**

Not answered

Marked out of 1

Which of the following drugs used to prevent alcohol relapse appears to act centrally on the glutamate and GABA neurotransmitter systems?

Select one:

- ☐ Chlormethiazole
- ☐ Acamprosate
- ☐ Disulfiram
- ☐ Carbamazepine
- ☐ Naltrexone

Your answer is incorrect.

Explanation:

Acamprosate is a derivative of the amino acid taurine and interacts with both the glutamate system to inhibit it and with the GABA system to enhance it, a bit like a form of 'artificial alcohol'. Thus, when alcohol is taken chronically and then withdrawn, the adaptive changes that it hypothetically causes in both the glutamate system and the GABA system create a state of glutamate overexcitement and even excitotoxicity as well as GABA deficiency. To the extent that acamprosate can substitute for alcohol in patients during withdrawal, the actions of acamprosate mitigate the glutamate hyperactivity and the GABA deficiency.

Ref: Stahl SM. Stahl's Essential Psychopharmacology: Neuroscientific Basis and Practical Applications, 5th Edition. 2021. p. 556.

The correct answer is: Acamprosate

Question 32

Not answered

Marked out of 1

Sally is 10 weeks pregnant. She is on 70mg methadone daily and wants to stop taking it. What would you advise her to do?

Select one:

- ☐ Stop immediately
- ☐ Switch to suboxone
- ☐ Switch to buprenorphine
- ☐ Reduce and stop over next 2 weeks
- ☐ Continue on treatment until delivery

Your answer is incorrect.

Explanation:

Women can present with opioid dependency at any stage in pregnancy, and stabilisation on substitute methadone is the treatment of choice. Both methadone and buprenorphine maintenance improve maternal and foetal outcomes, and substitution treatment should be offered to pregnant opioid-dependent women. Buprenorphine is associated with less severe neonatal withdrawal symptoms. However, buprenorphine should not be initiated in pregnancy because of the risk of inducing withdrawal in the foetus. Changing from methadone to buprenorphine is not recommended because of the risk of foetal withdrawal. There is little evidence for acute detoxification in pregnancy. Pregnant opioid-dependent women should be encouraged to access opioid maintenance treatment rather than attempt detoxification on the basis that relapse rates are high and maternal and foetal risks are greater from failed detoxification and relapse than from maintenance treatment.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 498-499.

McAllister-Williams RH et al. British Association for Psychopharmacology consensus guidance on the use of psychotropic medication preconception, in pregnancy and postpartum 2017. Journal of Psychopharmacology. 2017 May;31(5):519-52.

The correct answer is: Continue on treatment until delivery

Question 33

Not answered

Marked out of 1

Mr Y is a 55-year-old man who was admitted to the orthopaedic ward three days ago after sustaining a fracture to his right femur following a road traffic accident. He became confused, distracted and agitated in 2-3 days following surgery. Routine blood and urine tests do not indicate infection or blood loss. The most likely diagnosis is

Select one:

- ☐ Schizophrenia
- ☐ Schizophreniform psychosis
- ☐ Dementia
- ☐ Acute psychotic episode
- ☐ Delirium tremens

Your answer is incorrect.

Explanation:

Delirium tremens (DTs) is an acute confusional state secondary to alcohol withdrawal. It is often preceded by withdrawal symptoms; seizures may herald its onset. It occurs in approximately 5% of episodes of alcohol withdrawal. Risk factors for DTs include a long history of alcohol dependence, past history of DTs, being older, medically compromised, and poor nutritional state. DTs are characterised by a range of clinical symptoms including autonomic hyperactivity, marked agitation, clouding of consciousness, perceptual disturbances, and paranoid delusions. Mortality is 1-15%.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 590-591.

Sinclair JM, O'Neill A. Update on the management of alcohol use disorders. BJPsych Advances. 2020 Mar;26(2):82-91.

The correct answer is: Delirium tremens

Question **34**

Not answered

Marked out of 1

Following sudden cessation of alcohol in a dependent patient, the risk of seizure is high during which of the following time periods?

Select one:

- ☐ 14 days
- ☐ 3-5 days
- ☐ 7 days
- ☐ 24-48 hours
- ☐ First 24 hours

Your answer is incorrect.

Explanation:

Mild alcohol withdrawal symptoms usually start within 3-12 hours of the last drink and peak at 24-48 hours. They may last up to 14 days. Generalised seizures usually start within 12-18 hours of the last drink. They may commence before blood alcohol levels reach zero.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 454-455.

The correct answer is: First 24 hours



Question **35**

Not answered

Marked out of 1

A woman who repeatedly presents to A&E is pregnant and a heavy abuser of heroin. She plans to breastfeed following childbirth. Which medication is/are preferable to continue if she starts breastfeeding?

Select one:

- ☐ Naltrexone
- ☐ Lofexidine
- ☐ Buprenorphine
- ☐ Methadone
- ☐ Either buprenorphine or methadone

Your answer is incorrect.

Explanation:

Women prescribed methadone or buprenorphine should be encouraged to breastfeed even if they continue to use illicit opioids for the following reasons:

- General health benefits to the mother and the infant
- Specific benefits in reducing admission length and the need for intervention in neonatal abstinence syndrome
- Low concentrations of methadone and buprenorphine are transferred to the infant

Patients should be advised to discontinue breastfeeding gradually, as abrupt cessation can cause a delayed neonatal abstinence syndrome.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 499.

The correct answer is: Either buprenorphine or methadone

Question **36**

Not answered

Marked out of 1

After completing opioid detoxification, a man with a supportive partner is highly motivated to remain abstinent and attend follow-up appointments. Which of the following drugs is best suited in this situation?

Select one:

- ☐ Buprenorphine
- ☐ Methadone
- ☐ Naltrexone
- ☐ Disulfiram
- ☐ Naloxone

Your answer is incorrect.

Explanation:

Naltrexone has been found by NICE to be a cost-effective treatment strategy in aiding abstinence from opioid misuse for those who prefer an abstinence programme, are fully informed of the potential adverse effects and benefits of treatment, are highly motivated to remain on treatment, and have a partner supporting concordance.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 496.

The correct answer is: Naltrexone

Question **37**

Not answered

Marked out of 1

Tim is using heroin regularly. He has been tried unsuccessfully on methadone and then on buprenorphine. Choose the next best treatment option available in the UK.

Select one:

- ☐ Dihydrocodeine
- ☐ Clonidine
- ☐ Injectable diamorphine
- ☐ Slow-release oral morphine
- ☐ Combination of methadone and buprenorphine

Your answer is incorrect.

Explanation:

Oral options other than methadone and buprenorphine, such as slow-release oral morphine preparations and dihydrocodeine, are not licensed in the UK for the treatment of opioid dependence. There is compelling evidence supporting the use of injectable diamorphine maintenance prescribing for the treatment of patients who fail to benefit from first-line opioid substitution treatment (OST). In the UK, the prescribing doctor must have a license from the Home Office to prescribe diamorphine for opioid dependence. This treatment differs from 'injecting rooms', that is, safe places with sterile equipment for people who use IV drugs (usually not in treatment) in that it is part of a holistic package of care with adjunctive psychosocial interventions. It is an option in cases where the individual has not responded adequately to oral OST, and in an area where it can be supported by the necessary provisions for supervised consumption. Patients are generally seen for supervised injection in a specialist facility twice a day.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 490-491.

The correct answer is: Injectable diamorphine

Question **38**

Not answered

Marked out of 1

Tom is on methadone treatment for heroin addiction. He attends A&E on two occasions as he is "running out of methadone" and demands treatment. What is the best course of action?

Select one:

- ☐ Optimise the dose of methadone
- ☐ Combine methadone with buprenorphine
- ☐ Switch to suboxone
- ☐ Add dihydrocodeine
- ☐ Switch to buprenorphine

Your answer is incorrect.

Explanation:

During opioid maintenance treatment, the aim is to prevent under-dosing (risk of use of street opioids, withdrawal symptoms) and overdosing (sedation, diversion). It is possible that this man is experiencing withdrawal symptoms due to an inadequate dose of methadone and then self medicating. Patients like this should be assessed for withdrawal and, if present, methadone dosing should be adjusted accordingly.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 637.

The correct answer is: Optimise the dose of methadone

Question 39

Not answered

Marked out of 1

Sally is physically dependent on dihydrocodeine, which she is taking for low back pain. She wants to stop this and go back to work. What is the best course of action?

Select one:

- ☐ Commence her on clonidine
- ☐ Commence her on buprenorphine
- ☐ Withdraw codeine gradually
- ☐ Methadone substitution
- ☐ Prescribe suboxone

Your answer is incorrect.

Explanation:

There is a growing body of evidence that opioids are harmful in the treatment of low back pain. Opioids should never be prescribed long term and clinicians should try to **reduce and stop** opioids if they are not improving functioning. Other strategies to manage pain should be introduced. Non-pharmacological management options include active physiotherapy, patient-initiated stretching/strengthening, aerobic activity, yoga, pilates, and core-strengthening exercises targeting postural muscles. Exercise frequency has been shown to be more important than type, duration, or intensity of exercise.

Ref: Taylor DM, Gaughran F, Pillinger T. The Maudsley Practice Guidelines for Physical Health Conditions in Psychiatry. 2021. pp. 502-503.

The correct answer is: Withdraw codeine gradually

Question 40

Not answered

Marked out of 1

The risk of developing alcohol problems for those with a positive family history compared to those without a family history of alcohol problems is increased by

Select one:

- ☐ 2 times
- ☐ 10 times
- ☐ 6 times
- ☐ 4 times
- ☐ 8 times

Your answer is incorrect.

Explanation:

First-degree relatives of alcoholics have double the risk of alcohol problems themselves.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 580.

Haugland SH et al. Associations between parental alcohol problems in childhood and adversities during childhood and later adulthood: a cross-sectional study of 28047 adults from the general population. Substance Abuse Treatment, Prevention, and Policy. 2021 Dec;16(1):1-7.

The correct answer is: 2 times



Question **41**

Not answered

Marked out of 1

A patient is keen to stop heroin use but is anxious about experiencing withdrawal symptoms. Which of the following is best suited to support detoxification in this patient?

Select one:

- ☐ Buprenorphine
- ☐ Lofexidine
- ☐ Methadone
- ☐ Loperamide
- ☐ Clonidine

Your answer is incorrect.

Explanation:

Of particular interest in the initiation of buprenorphine is the phenomenon of precipitated withdrawal. This occurs because buprenorphine is a partial agonist with a high receptor affinity. If it enters the brain when a full agonist (e.g. methadone or heroin) is still present, it competes for binding at the opioid receptors and replaces the full agonist. Therefore, some receptors that were previously fully stimulated become partially stimulated and the patient experiences this change as opioid withdrawal. Methadone, as a full agonist, does not produce precipitated withdrawal and may be a better option for patients who are anxious about withdrawal symptoms.

Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 486.

The correct answer is: Methadone

Question 42

Not answered

Marked out of 1

What percentage of patients with schizophrenia use nicotine?

Select one:

- ☐ 70-80%
- ☐ 90-100%
- ☐ 33-43%
- ☐ 14-24%
- ☐ 20-25%

Your answer is incorrect.

Explanation:

Nicotine is consumed by vaping or tobacco smoking. 70-80% of those with schizophrenia regularly smoke cigarettes (with increasing numbers switching to vaping). Possible explanations for higher rates of nicotine use are as follows: smoking causes dopamine release, leading to feelings of well-being and a reduction in negative symptoms; smoking alleviates some of the side effects of antipsychotics such as drowsiness and extrapyramidal symptoms and cognitive slowing; smoking serves as a means of structuring the day (a behavioural filler); smoking arises as a result of familial vulnerability; and/or smoking may be used as a means of alleviating the deficit in auditory gating that is found in schizophrenia.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 871-872.

The correct answer is: 70-80%

Question 43

Not answered

Marked out of 1

A 35-year-old man states that he is currently drinking two bottles of wine a day. He is unable to cut down due to symptoms of insomnia, agitation and tremors. He is also reported to suffer from low mood, anhedonia and fatiguability. What would be the most appropriate treatment option?

Select one:

- ☐ Mirtazapine
- ☐ Disulfiram
- ☐ Alcohol detoxification
- ☐ Cognitive behavioural therapy
- ☐ Zopiclone

Your answer is incorrect.

Explanation:

Alcohol withdrawal often takes place without medical or pharmacological treatment. Alcohol withdrawal is associated with significant morbidity and mortality when improperly managed. Pharmacological assisted withdrawal (alcohol detoxification) is likely to be needed under the following circumstances: regular consumption > 15 units/day; AUDIT score > 20; there is a history of significant withdrawal symptoms.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 456.

The correct answer is: Alcohol detoxification

Question **44**

Not answered

Marked out of 1

Which of the following is a mechanism of action of acamprosate in treating alcohol dependence?

Select one:

- ☐ Reducing GABA transmission
- ☐ Increasing neuronal excitability
- ☐ Balancing the GABA and glutamate systems
- ☐ Increasing glutamate transmission
- ☐ Membrane stabilization

Your answer is incorrect.

Explanation:

Acamprosate is a derivative of the amino acid taurine and interacts with both the glutamate system to inhibit it and with the GABA system to enhance it, a bit like a form of 'artificial alcohol'. Thus, when alcohol is taken chronically and then withdrawn, the adaptive changes that it hypothetically causes in both the glutamate system and the GABA system create a state of glutamate overexcitement and even excitotoxicity as well as GABA deficiency. To the extent that acamprosate can substitute for alcohol in patients during withdrawal, the actions of acamprosate mitigate the glutamate hyperactivity and the GABA deficiency.

Ref: Stahl SM. Stahl's Essential Psychopharmacology: Neuroscientific Basis and Practical Applications, 5th Edition. 2021. p. 556.

The correct answer is: Balancing the GABA and glutamate systems

Question 45

Not answered

Marked out of 1

A 55-year-old man with a history of opioid dependence would like to consider pharmacological maintenance treatment along with psychosocial interventions. Of the listed options, the most appropriate medication would be

Select one:

- ☐ Suboxone
- ☐ Naltrexone
- ☐ Buprenorphine
- ☐ Lofexidine
- ☐ Diamorphine

Your answer is incorrect.

Explanation:

The most recent NICE guidelines for the management of opioid dependence (2007, update 2016) recommend **methadone** and **buprenorphine** as options for maintenance therapy. The decision about which drug to use should be made on a case-by-case basis.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on methadone and buprenorphine for the management of opioid dependence. Updated edition 2016.

The correct answer is: Buprenorphine



Question 46

Not answered

Marked out of 1

Which of the following screening tools has only four questions?

Select one:

- ☐ Minnesota Multiphasic Inventory
- ☐ Clinical Alcohol Withdrawal Scale
- ☐ Alcohol Use Disorders Identification Test
- ☐ CAGE questionnaire
- ☐ Michigan Alcohol Screening Test

Your answer is incorrect.

Explanation:

The **CAGE questionnaire** is a brief screening tool used for the identification of at-risk drinking. It consists of **4 questions**: Have you ever felt you should **Cut** back on your drinking? Has anyone ever **Annoyed** you by criticizing your drinking? Have you ever felt **Guilty** about your drinking? Have you ever had a drink early in the morning as an **Eye-opener**? More than 2 positive responses suggest **possible at-risk drinking** and should prompt further assessment. The CAGE offers no objective assessment of the frequency or quantity of alcohol use.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 582.

The correct answer is: CAGE questionnaire

Question **47**

Not answered

Marked out of 1

Which of the following substances is an active ingredient of the Ethiopian plant *Catha edulis*, whose leaves on chewing provide amphetamine-like stimulation?

Select one:

- ☐ Cathine
- ☐ Coumarin
- ☐ Ketamine
- ☐ Cathinone
- ☐ Ephedrine

Your answer is incorrect.

Explanation:

Fresh leaves of *Catha edulis* (or 'Khat'), a bush native to East Africa, are used as a stimulant for their amphetamine-like effects induced by the main ingredient, cathinone. Cathinone has most of the CNS and peripheral actions of amphetamine and appears to have the same mechanism of action. Khat is chewed like tobacco and held between the cheek and gums after chewing. Because the alkaloid is metabolized rapidly, high toxic blood levels are rarely reached. Synthetic cathinones are available for illicit use in many countries.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 303.

The correct answer is: Cathinone

Question **48**

Not answered

Marked out of 1

Mr. P has a long standing history of substance misuse and has experienced multiple strokes. Which of the following substances is most likely to have precipitated the strokes?

Select one:

- ☐ Phencyclidine
- ☐ Anabolic steroids
- ☐ Heroin
- ☐ Cannabis
- ☐ Cocaine

Your answer is incorrect.

Explanation:

Stimulants such as cocaine are potent sympathomimetic drugs, causing physiological effects, including vasoconstriction, increased heart rate, and elevated blood pressure through adrenergic stimulation. The combination of increased myocardial demand with constriction of coronary blood vessels can precipitate angina pectoris and even myocardial infarction. Acute hypertension associated with stimulant use can also predispose to arterial haemorrhages, such as haemorrhagic stroke or aortic dissection. Vasoconstriction can involve damage to other end organs, including the brain, kidneys, or intestines. Grand mal seizures are frequent with stimulant overdose and may occur with routine use as well.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 307.

The correct answer is: Cocaine

Question 49

Not answered

Marked out of 1

Which of the following is least suggestive of an opioid withdrawal state?

Select one:

- ☐ Constricted pupils
- ☐ Piloerection
- ☐ Insomnia
- ☐ Hypertension
- ☐ Tachycardia

Your answer is incorrect.

Explanation:

Pupillary dilation, not constriction, is seen in opioid withdrawal. Other opioid withdrawal symptoms include dysphoric mood, nausea or vomiting, muscle aches, lacrimation or rhinorrhea, piloerection or gooseflesh (from which comes the term 'cold turkey' for the abstinence syndrome), diarrhoea, yawning, fever, insomnia, hypertension, and tachycardia. Persons with opioid dependence seldom die from opioid withdrawal unless they have a severe pre-existing physical illness such as cardiac disease. Residual symptoms – such as insomnia, bradycardia, temperature dysregulation, and a craving for opioids – can persist for months after withdrawal.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 292.

The correct answer is: Constricted pupils

Question 50

Not answered

Marked out of 1

A 45-year-old man with a harmful alcohol habit is ambivalent about making any changes. He is in which stage of change?

Select one:

- ☐ Preparation
- ☐ Maintenance
- ☐ Contemplation
- ☐ Pre-contemplation
- ☐ Action

Your answer is incorrect.

Explanation:

Six stages of change:

- (1) **Precontemplation:** not even considering changing behaviour; does not see the behaviour as a problem, minimizes and denies associated risks, and avoids information to the contrary
- (2) **Contemplation:** has become aware that the behaviour is a problem but is ambivalent about changing, and likely sees equal or more benefits than costs from the behaviour
- (3) **Preparation:** has made a decision to attempt change
- (4) **Action:** has implemented a plan and is changing the behaviour
- (5) **Maintenance:** maintaining gains made and attempting to improve areas of life that have been harmed
- (6) **Relapse:** return to previous behaviour, but with the possibility of gaining useful strategies to extend the maintenance period on the next attempt

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 575.

The correct answer is: Contemplation

Question **51**

Not answered

Marked out of 1

Which of the following is most common during cocaine withdrawal?

Select one:

- ☐ Delusions
- ☐ Heightened energy
- ☐ Hallucinations
- ☐ Decreased appetite
- ☐ Dysphoric mood

Your answer is incorrect.

Explanation:

Features of cocaine withdrawal may include dysphoric mood ('crash') sometimes with suicidal ideation, irritability, fatigue, vivid & unpleasant dreams, insomnia or (more commonly) hypersomnia, increased/insatiable appetite, psychomotor agitation or retardation, and craving for cocaine.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

The correct answer is: Dysphoric mood

Question 52

Not answered

Marked out of 1

In Korsakoff syndrome, the type of memory that is most affected is

Select one:

- ☐ Episodic memory
- ☐ Semantic memory
- ☐ Implicit memory
- ☐ Procedural memory
- ☐ Working memory

Your answer is incorrect.

Explanation:

Korsakoff syndrome is a largely irreversible residual syndrome, caused by severe thiamine deficiency and occurring after incomplete recovery from a Wernicke encephalopathy, predominantly in the context of alcohol abuse and malnutrition, characterised by an abnormal mental state in which **episodic memory** is affected out of proportion to other cognitive functions in an otherwise alert and responsive patient, whose psychological make-up may be further distinguished by executive dysfunction, flattened affect, apathy, lack of illness insight, and possibly by fantastic confabulations in the early stage. Working memory (e.g. ability to remember a sequence of numbers) is unimpaired, as is procedural and 'emotional' memory. Thus, the affected individual may be able to go with a psychologist to an interview room, perform adequately on working memory testing, show evidence of a new skill (e.g. mirror writing) they practised the day before, and yet later have no memory of ever having been in that room or having seen that psychologist before.

Ref: Arts NJ et al. Korsakoff's syndrome: a critical review. Neuropsychiatric disease and treatment. 2017 Nov 27.

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 606-607.

The correct answer is: Episodic memory

Question 53

Not answered

Marked out of 1

Which of the following statements is true regarding ICD-11's 'harmful pattern of use' compared to 'hazardous substance use'?

Select one:

- ☐ Harmful pattern of use requires a time period of 12 months, whereas hazardous pattern of use requires a time period of 1 month
- ☐ In harmful pattern of use, damage has been caused to the user or others, whereas no harm has occurred in hazardous use
- ☐ In both harmful pattern of use and hazardous use, harm has been caused to the user but not to others
- ☐ A user can be diagnosed with both harmful pattern of use and hazardous use of the same substance simultaneously
- ☐ Once users with hazardous use become aware of the risks, they quit using, whereas users with a harmful pattern continue to use

Your answer is incorrect.

Explanation:

ICD-11 has a diagnostic code for '**harmful pattern of use**' wherein the pattern of substance use has caused damage to a person's physical or mental health or has resulted in behaviour leading to harm to the health of others. This pattern of use is evident over a period of at least 12 months if substance use is episodic or at least one month if use is continuous.

'**Hazardous substance use**' is a pattern of psychoactive substance use that **appreciably increases the risk** of harmful physical or mental health consequences to the user or to others to an extent that warrants attention and advice from health professionals but **has not resulted in specific identifiable harm** and therefore does **not** meet the diagnostic requirements for harmful pattern of use. Hazardous substance use often persists in spite of awareness of increased risk of harm to the user or others. There is no time period associated with hazardous substance use.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

The correct answer is: In harmful pattern of use, damage has been caused to the user or others, whereas no harm has occurred in hazardous use

Question **54**

Not answered

Marked out of 1

The route of cocaine administration that can produce a 'high' that can last up to 15 to 30 minutes is

Select one:

- ☐ Intranasal application
- ☐ Sublingual application
- ☐ Injecting intravenously
- ☐ Smoking
- ☐ Oral ingestion

Your answer is incorrect.

Explanation:

The duration of cocaine's euphoric effects ('high') depends upon the route of administration. The faster the absorption, the more intense the 'high' but the shorter the duration. The high from snorting (intranasal application) is relatively slow in onset and may last 15 to 30 minutes, while that from smoking is more immediate but may last only 5 to 10 minutes.

Ref: <https://www.drugabuse.gov/publications/research-reports/cocaine/what-are-short-term-effects-cocaine-use>

The correct answer is: Intranasal application

Question 55

Not answered

Marked out of 1

A man who has been abusing multiple substances now complains of frequent urination and passing blood in the urine. The most likely offending drug is

Select one:

- ☐ Cannabis
- ☐ Heroin
- ☐ Crack
- ☐ LSD
- ☐ Ketamine

Your answer is incorrect.

Explanation:

Ketamine is structurally similar to phencyclidine and is used as an anaesthetic. It is a unique anaesthetic, as it does not depress the reticular activating system (thus does not usually knock out consciousness completely) but reduces the cortical awareness of painful stimuli. It is taken illicitly as a sniffed powder. The use of ketamine can cause urinary tract symptoms (e.g. frequency, urgency, urge incontinence, dysuria, and haematuria). The causal agent has not been determined, but direct irritation by ketamine and/or its metabolites is a possibility. Investigations have revealed interstitial cystitis, detrusor overactivity, and decreased bladder capacity. Symptoms generally settle several weeks after stopping ketamine.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 625.

Stahl SM. Stahl's Essential Psychopharmacology: Prescriber's Guide, 7th Edition. 2021. pp. 386.

The correct answer is: Ketamine

Question 56

Not answered

Marked out of 1

Globally, stimulant (cocaine and amphetamines) dependence accounts for a significant proportion of what cause of death?

Select one:

- ☐ Suicide
- ☐ Accidental injuries
- ☐ Cardiovascular disease
- ☐ Homicide
- ☐ All of the listed options

Your answer is incorrect.

Explanation:

Globally, stimulant dependence accounts for an important number of suicides, accidental injuries, cardiovascular disease, and homicide deaths. Cocaine dependence was associated with 0.65% of suicide deaths, 0.24% of accidental injury deaths, 0.14% of cardiovascular deaths, and 0.47% of homicide deaths in 2017. Amphetamine dependence was associated with 1.23% of suicide deaths, 0.59% of accidental injury deaths, 0.48% of cardiovascular deaths, and 1.23% of homicide deaths in 2017.

Ref: Farrell M et al. Responding to global stimulant use: challenges and opportunities. The Lancet. 2019 Nov 2;394(10209):1652-67.

Butler AJ et al. Health outcomes associated with crack-cocaine use: Systematic review and meta-analyses. Drug and Alcohol Dependence. 2017 Nov 1;180:401-16.

The correct answer is: All of the listed options

Question **57**

Not answered

Marked out of 1

Among the following, the drug least likely to cause physical or psychological dependence is

Select one:

- ☐ Cocaine
- ☐ Cannabis
- ☐ LSD
- ☐ Amphetamine
- ☐ Alcohol

Your answer is incorrect.

Explanation:

LSD carries no risk of overdose, and physiological dependence and withdrawals do **not** occur.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 624.

The correct answer is: LSD



Question 58

Not answered

Marked out of 1

A pregnant woman in her 2nd trimester is insistent on undergoing opioid detoxification. Which one of the following is the suitable medication?

Select one:

- ☐ Naloxone
- ☐ Carbamazepine
- ☐ Naltrexone
- ☐ Methadone
- ☐ Buprenorphine

Your answer is incorrect.

Explanation:

Women can present with opioid dependency at any stage in pregnancy, and stabilisation on substitute methadone is the treatment of choice. Detoxification in the first trimester is contraindicated due to the risk of spontaneous abortion and in the third trimester it is associated with preterm delivery, foetal distress, and stillbirth. If detoxification is requested, this is most safely achieved in the second trimester but should only be supervised by specialists with the appropriate competencies and with careful monitoring for any evidence of instability. Detoxification should be prescribed in small frequent decrements (e.g. 2-3mg of methadone every 3-5 days). Enforcing detoxification is contraindicated.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 498.

The correct answer is: Methadone

Question 59

Not answered

Marked out of 1

You are asked to see a 24-year-old man in the A&E. He is attempting to withdraw from heroin. Which of the following signs suggests an advanced opioid withdrawal state?

Select one:

- ☐ Increased respiratory rate
- ☐ Diarrhoea
- ☐ Muscle spasm
- ☐ Piloerection
- ☐ Headache

Your answer is incorrect.

Explanation:

Opioid withdrawal symptoms may include lacrimation, rhinorrhoea, agitation, perspiration, piloerection, vomiting, shivering, yawning and dilated pupils. Muscle spasms and twitching are signs of advanced opioid withdrawal state, and get higher scores on the Clinical Opiate Withdrawal Scale.

Ref: <https://elsevier.health/en-US/preview/opioid-withdrawal>

The correct answer is: Muscle spasm



Question 60

Not answered

Marked out of 1

In patients using recreational drugs, which of the following features is characteristic of phencyclidine intoxication?

Select one:

- ☐ Respiratory depression
- ☐ Peripheral analgesia
- ☐ Hypothermia
- ☐ Dilatation of pupils
- ☐ Hypotension

Your answer is incorrect.

Explanation:

PCP is a hallucinogen rarely seen in the UK, except as a contaminant of other drugs. It may be smoked, snorted, taken orally, or, more rarely parenterally. Features of PCP intoxication may include aggression, impulsiveness, unpredictability, anxiety, psychomotor agitation, impaired judgement, **numbness or diminished responsiveness to pain**, slurred speech, and dystonia. Physical signs include nystagmus, tachycardia, elevated blood pressure, **numbness**, ataxia, dysarthria, and muscle rigidity. In rare instances, seizures may occur.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 624.

International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

The correct answer is: Peripheral analgesia

Question **61**

Not answered

Marked out of 1

Which of the following is correct with respect to the use of LSD?

Select one:

- ☐ Physiological withdrawal is not seen
- ☐ It is available as a prescribed drug
- ☐ Onset of action is delayed for 2 hours in most subjects
- ☐ Frequent use commonly causes tolerance
- ☐ It is often injected

Your answer is incorrect.

Explanation:

LSD is a synthetic hallucinogen synthesized in 1944 by Albert Hoffman. There was early experimentation with its role in psychotherapy, but there is no current legitimate use. It is sold impregnated onto paper, in tablets, or as a powder. Effects develop 15-30 mins after ingestion and last up to 6 hours. There is no risk of OD, and physiological dependence and withdrawals do not occur.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. P. 624.

The correct answer is: Physiological withdrawal is not seen

Question 62

Not answered

Marked out of 1

Identify the correct statement regarding treatment of opioid misuse with naltrexone?

Select one:

- ☐ Fatal overdose in those who relapse is rare
- ☐ It has a much shorter half life than naloxone
- ☐ Prior to giving the drug, patients should be advised of the risk of withdrawal
- ☐ Combining naltrexone with psychological interventions provides limited additional benefit over naltrexone alone
- ☐ Its effectiveness as a treatment for relapse prevention in opioid misusers is confirmed

Your answer is incorrect.

Explanation:

Evidence for the effectiveness of naltrexone as a treatment for relapse prevention in opioid misusers has been inclusive. However, naltrexone has been found by NICE to be a cost-effective treatment strategy in aiding abstinence from opioid misuse for those who prefer an abstinence programme, are fully informed of the potential adverse effects and benefits of treatment, are highly motivated to remain on treatment, and have a partner supporting concordance. Close monitoring is particularly important when naltrexone is initiated because of the higher risk of fatal overdose at this time. Discontinuation of naltrexone may also be associated with an increase in inadvertent overdose from illicit opioids. People taking naltrexone often experience dysphoria, which may be caused by either withdrawal from illicit drugs or by the naltrexone itself. This emphasises the importance of prescribing naltrexone as part of a care programme that includes psychosocial therapy and general support. Naltrexone has the propensity to cause a severe withdrawal reaction in patients who are either currently taking opioid drugs or who were previously taking opioid drugs and there has not been a sufficient 'wash-out' period. A test dose of naloxone, which has a much shorter half-life than naltrexone, may be given to the patient as an IM dose prior to starting naltrexone treatment. Patients must be advised of the risk of withdrawal before taking naltrexone.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 496.

The correct answer is: Prior to giving the drug, patients should be advised of the risk of withdrawal

Question 63

Not answered

Marked out of 1

In phencyclidine intoxication, which of the following is least likely to be seen?

Select one:

- ☐ Dysarthria
- ☐ Nystagmus
- ☐ Ataxia
- ☐ Psychomotor retardation
- ☐ Rigidity

Your answer is incorrect.

Explanation:

PCP is a hallucinogen rarely seen in the UK, except as a contaminant of other drugs. It may be smoked, snorted, taken orally, or, more rarely parenterally. People who have just taken PCP are frequently uncommunicative, appear to be oblivious, and report active fantasy production. PCP intoxication can cause vertical or horizontal nystagmus, numbness or diminished responsiveness to pain, ataxia, dysarthria, muscle rigidity, and repeated episodes of vomiting. Occasionally, users become belligerent, irrationally assaultive, suicidal, or homicidal. Seizures are very rare.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 624.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 320.

The correct answer is: Psychomotor retardation

Question **64**

Not answered

Marked out of 1

Which of the following is not a feature of opioid withdrawal?

Select one:

- ☐ Lacrimation
- ☐ Tachycardia
- ☐ Piloerection
- ☐ Pupillary constriction
- ☐ Diarrhoea

Your answer is incorrect.

Explanation:

Pupillary dilation, not constriction, is seen in opioid withdrawal. Other opioid withdrawal symptoms include dysphoric mood, nausea or vomiting, muscle aches, lacrimation or rhinorrhea, piloerection or gooseflesh (from which comes the term 'cold turkey' for the abstinence syndrome), diarrhoea, yawning, fever, insomnia, hypertension, and tachycardia. Persons with opioid dependence seldom die from opioid withdrawal unless they have a severe pre-existing physical illness such as cardiac disease. Residual symptoms – such as insomnia, bradycardia, temperature dysregulation, and a craving for opioids – can persist for months after withdrawal.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 292.

The correct answer is: Pupillary constriction

Question 65

Not answered

Marked out of 1

Which of the following could be used as a replacement therapy for a heroin user?

Select one:

- ☐ Codeine
- ☐ Lofexidine
- ☐ Suboxone
- ☐ Naloxone
- ☐ Naltrexone

Your answer is incorrect.

Explanation:

Suboxone is a combination preparation of buprenorphine with naloxone. This preparation may reduce the risk of diversion. The different sublingual and parenteral absorption profiles of buprenorphine and naloxone are the key factors: if used sublingually, the naloxone will have negligible effects. However, if the combined preparation is injected, the naloxone will have a substantial effect and will attenuate the effects of buprenorphine in the short term and is also likely to precipitate withdrawal in opioid-dependent individuals on full opioid agonists.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 490.

The correct answer is: Suboxone

Question 66

Not answered

Marked out of 1

A 36-year-old cocaine user notices that the longer he uses the drug, the more of it is needed to achieve the same effect. This is best described as

Select one:

- ☐ Dependence
- ☐ Tolerance
- ☐ Compliance
- ☐ Sensitisation
- ☐ Addiction

Your answer is incorrect.

Explanation:

When tolerance develops, the user finds that more of the substance must be taken to achieve the same effects. Tolerance is exhibited in individuals who display no or few signs of intoxication while at a blood level in which intoxication would be evident in a non-dependent individual. In the case of drugs, the user may attempt to combat increasing tolerance by using a more rapidly acting route of administration or by choosing a more rapidly acting form of the drug.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 574.

Edwards G, Gross MM. Alcohol dependence: provisional description of a clinical syndrome. British medical journal. 1976 May 1;1(6017):1058.

The correct answer is: Tolerance

Question **67**

Not answered

Marked out of 1

Mr. Y is a regular drug user who presents to the A&E experiencing vivid, unpleasant dreams. He is most likely to be experiencing withdrawal from which of the following substances?

Select one:

- ☐ Alcohol
- ☐ Cannabis
- ☐ LSD
- ☐ Amphetamine
- ☐ Morphine

Your answer is incorrect.

Explanation:

Features of stimulant withdrawal (including amphetamines, methamphetamine, and methcathinone) may include dysphoric mood ('crash') sometimes with suicidal ideation, irritability, fatigue, vivid & unpleasant dreams, insomnia or (more commonly) hypersomnia, increased/insatiable appetite, psychomotor agitation or retardation, and craving for amphetamine or related stimulants.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

The correct answer is: Amphetamine

Question 68

Not answered

Marked out of 1

A 60-year-old woman with a long standing history of alcoholism presents with ataxia, nystagmus and confusion. She is admitted to the medical ward. Which one among the following drugs should be used in the management of this case?

Select one:

- ☐ Oral Vitamin B6
- ☐ Intravenous Vitamin B12
- ☐ Oral Vitamin B1
- ☐ Oral Vitamin B12
- ☐ Intravenous Vitamin B1

Your answer is incorrect.

Explanation:

Wernicke encephalopathy is an acute neuropsychiatric condition caused by thiamine deficiency. If Wernicke encephalopathy is suspected, the patient should be transferred to a medical unit where IV thiamine can be administered. At least two pairs of Pabrinex (parenteral B1 replacement) IV high potency (i.e. 4 ampoules) should be given 3 times daily for 3-5 days, followed by one pair of ampoules once daily for a further 3-5 days or longer (until no further response is seen).

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 461-462.

Sample D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 606.

The correct answer is: Intravenous Vitamin B1

Question 69

Not answered

Marked out of 1

Which of the following is true about the pharmacokinetics of alcohol?

Select one:

- ☐ Females have less risk of intoxication for the same units consumed when compared to males
- ☐ Drinks with more than 30% alcohol content are absorbed more readily than those with less concentration
- ☐ Reduced gastric motility increases blood alcohol levels
- ☐ Most alcohol is excreted unchanged in urine
- ☐ Gastrectomy increases the absorption of alcohol

Your answer is incorrect.

Explanation:

The longer alcohol is exposed to gastric alcohol dehydrogenase, the greater will be its breakdown (and consequently less the blood levels). As a result, conditions that reduce gastric stay of alcohol (e.g., surgical gastrectomy) increases the chances of a rapid overshoot of alcohol in blood and intoxication. On the other hand, conditions that increase gastric stay (e.g. pyloric stenosis) may reduce acute intoxication. In fact, an increase in alcohol-related disease occurs after bariatric surgery, likely in relation to this effect.

Most alcohol is broken down by the liver (85%). A small proportion (2-5%) of the alcohol absorbed is excreted unchanged in the urine, sweat or breath. Females have a higher risk of intoxication as they have only 50% of alcohol dehydrogenase that is seen in the gastric mucosa of men. Drinks with an alcohol content between 20-30% are absorbed quickest. Drinks with a further high alcohol content are absorbed more slowly as an alcohol content over 30% irritates the gastric mucosa, stimulates mucus secretion and reduces gastric emptying.

Ref: Seyedsadjadi N, Acevedo MB, Alfaro R, Ramchandani VA, Plawecki MH, Rowitz B, Pepino MY. Site of Alcohol First-Pass Metabolism Among Women. JAMA network open. 2022 Mar 1;5(3):e223711-.

The correct answer is: Gastrectomy increases the absorption of alcohol

Question 70

Not answered

Marked out of 1

Which of the following signs is highlighted in the ICD-11 for a diagnosis of hallucinogen intoxication?

Select one:

- ☐ Tachycardia
- ☐ Vomiting
- ☐ Tachypnea
- ☐ Conjunctival injection
- ☐ Miosis

Your answer is incorrect.

Explanation:

Features of hallucinogen intoxication may include hallucinations, illusions, perceptual changes such as depersonalisation, derealisation, or synaesthesias, anxiety, depressed or dysphoric mood, ideas of reference, paranoid ideation, impaired judgement, palpitations, sweating, blurred vision, tremors, and incoordination. Physical signs may include tachycardia, elevated blood pressure, and pupillary dilatation. In rare instances, hallucinogen intoxication may facilitate suicidal ideation and behaviour.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

The correct answer is: Tachycardia

Question 71

Not answered

Marked out of 1

What is the maximum amount of alcohol per week that is advisable for a woman who is pregnant?

Select one:

- ☐ 1-2 units/week
- ☐ 6-8 units/week
- ☐ 4-5 units/week
- ☐ 0 units/week
- ☐ 3-4 units/week

Your answer is incorrect.

Explanation:

New revised low-risk drinking guidelines were published UK-wide in 2016. They proposed that to minimise health risks from alcohol, a limit of 14 units per week for both men and women who drink regularly is recommended. This constituted a reduction for men, the previous limit being 21 units per week. For those drinking up to 14 units, spreading drinking over 3 or more days is advised. Drink-free days per week are also recommended. No alcohol during pregnancy is advised as the safest approach.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 586.

The correct answer is: 0 units/week

Question 72

Not answered

Marked out of 1

Which of the following statements about alcoholic hallucinosis is incorrect?

Select one:

- ☐ Patients with alcoholic hallucinosis have a clear sensorium
- ☐ It usually occurs in persons abusing alcohol for a long time
- ☐ The hallucinations usually resolve rapidly if alcohol consumption stops
- ☐ The most common hallucinations are unstructured sounds or voices that may be derogatory
- ☐ The hallucinations cannot be treated with antipsychotics

Your answer is incorrect.

Explanation:

Alcoholic hallucinosis is a substance-induced psychotic illness that is a rare complication of prolonged heavy alcohol use. In ICD-11, it is designated as 'alcohol-induced psychotic disorder with hallucinations'. It is characterised by experiencing hallucinations (usually auditory) in clear consciousness and while sober. It usually starts with elemental hallucinations (e.g. bangs or murmurings). With continued alcohol use they become formed voices, usually derogatory. In 95% of patients, symptoms rapidly resolve on ceasing alcohol consumption, but rapidly recur on restarting drinking. In approximately 5%, there are prolonged symptoms. Persisting symptoms can be treated with antipsychotics.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 602.

International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

The correct answer is: The hallucinations cannot be treated with antipsychotics

Question **73**

Not answered

Marked out of 1

A man with an alcohol use disorder believes that his wife is cheating on him despite the absence of any evidence supporting his claim. This is a recognised feature of

Select one:

- ☐ Othello syndrome
- ☐ Cotard syndrome
- ☐ De Clerambault syndrome
- ☐ Capgras syndrome
- ☐ Couvade syndrome

Your answer is incorrect.

Explanation:

Pathological jealousy, or Othello syndrome, is a monosymptomatic delusional disorder. It is most commonly seen secondary to current or previous alcohol abuse. The content of the primary delusion is that the patient's spouse or partner has been, or is being, unfaithful. The patient may go to great lengths to obtain 'evidence'. There is a significant association with violence and even homicide towards supposedly unfaithful partner. It may be necessary for the couple to separate, and this advice may need to be given to the at-risk partner.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 603.

The correct answer is: Othello syndrome



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